A clinic front desk and care workflow, in one place

How patients move from first contact to follow-up, what doctors and administrators do, and how the system can become more automated.

The idea in one minute

Sahayak gives a small clinic a shared operational layer around the consultation. It can handle appointment enquiries, check-in, the live queue, routine patient conversations and approved follow-up workflows. Doctors retain control of clinical decisions and sign-off. Reception supervises exceptions. Admin configures the clinic, team and communication connections.

The complete journey

1. Discover and book	A patient asks about the clinic or requests an appointment. The assistant shows real availability and saves a booking only after explicit confirmation.
2. Arrive and wait	The patient checks in with consent, receives a token and joins a live queue. Intake answers can be collected while waiting.
3. Consult	The doctor sees a pre-visit brief, calls the next patient, documents the visit, reviews drafts and signs clinical material.
4. Follow through	Approved instructions, tasks and check-ins are tracked. Patient replies and concerns return to a human owner.
5. Improve	The clinic reviews service feedback, outstanding work and measured activity to improve operations.

Current product state: the repository contains working local screens, clinic-scoped data, simulated messaging, doctor workflows and several provider bridges. Live patient delivery and calling have separate readiness gates, described later.

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01 Who uses Sahayak

Three staff workspaces and patient-facing entry points support one clinic workflow. Each staff member signs in with a role-specific account.

Role	What they do
Doctor	Runs consultations; sees Today, patients, care and clinic views; reviews inbox items, notes, results, tasks, instructions and check-ins; signs or approves clinical output.
Receptionist	Runs appointments and queue; manually checks in patients; handles conversations, handoffs, callbacks, waiting-list offers and privacy requests.
Admin	Sets clinic hours and information, creates doctor and receptionist logins, configures assistants, communication connections and check-in message settings.
Patient	Can use a clinic link or QR check-in, a Hindi/English conversation, a waiting-room display, and private post-visit feedback/check-in forms.

Operating model

- **Sahayak owns the clinic record:** patients, consent, bookings, queue, signed documents, care status and audit history.
- **Assistants use approved tools:** they cannot directly change the database. Tool calls validate permissions and leave an audit trail.
- **Humans retain clinical authority:** the doctor reviews and signs notes, instructions, plans and clinical resolutions. The assistant does not diagnose or prescribe.
- **Reception handles exceptions:** handoff requests, unresolved conversations and operational follow-through remain visible to staff.

A typical day

Before clinic: Admin maintains hours, staff accounts and connections. During clinic: Reception oversees arrivals and messages while the doctor works through patients and urgent items. After visits: the doctor approves next steps; staff tracks replies and follow-up until a human records the outcome.

02 Patient journey: step by step

Step	What happens
1	**Patient contacts the clinic.** The local simulator or a connected channel receives a message. Safety and privacy checks run before the appointment menu.
2	**Assistant answers or books.** It can provide configured hours/directions, ask first-visit or follow-up, identify a doctor if needed, collect a preferred time, show actual open slots, and request a final YES.
3	**Booking is saved.** A slot is rechecked at confirmation so stale choices are not silently booked. Reminder consent is separate from booking.
4	**Patient checks in.** A clinic QR/link collects minimal details and consent, recognizes a returning patient by phone, issues a token, and updates the clinic queue.
5	**Patient waits.** The waiting-room screen shows now serving, upcoming tokens and an estimated wait. The doctor can call the next person; the patient can receive a turn notice where the channel is configured.
6	**Pre-visit intake.** Standard questions gather the reason for visit. The doctor can see source-linked answers in a pre-visit brief.
7	**Consultation.** The doctor reviews the patient record, dictates or types the note, edits the draft and signs it. The doctor can create documents, instructions and care tasks.
8	**After the visit.** The doctor may approve a follow-up plan or schedule an agreed check-in. Patient responses become staff work items; unresolved concerns stay open.

If something needs human attention

Emergency phrases trigger a fixed emergency response and urgent staff alert. Medical questions and requests to speak with a person are handed to the clinic. A failed AI service does not stop the basic check-in and queue workflow.

03 How a doctor uses it

Start of the day

- Open **Today** for appointments, live queue and a concise count of work needing attention.
- Open the **inbox** for urgent alerts, patient conversations, overdue follow-up, unreviewed reports, unsigned notes and draft documents. Assign or acknowledge work; defer nonurgent items when appropriate.
- Search for a patient and read the **pre-visit brief**: booking concern, intake answers, previous visits, signed notes, uploaded files, verified medicines/allergies/diagnoses and outstanding work, each linked to a source.

During the consultation

- Call the next patient, or hold/skip according to the queue situation. The live queue and waiting display update.
- Type or dictate a note. The composer preserves the original words and offers structured sections for history, assessment, differential, investigations and instructions. The doctor edits and signs.
- Create or review prescriptions, certificates, reports/referral tasks and patient instructions. Drafts are not final until the doctor signs or approves them.
- Use the finish checklist to see unsigned notes, document drafts, unverified data, pending instructions and tasks before closing the visit.

After the consultation

- Approve patient-specific follow-up steps with an owner and due date. Record contact, patient reply and doctor review; urgent replies open alerts.
- Review uploaded reports and structured values against their original source. Outside-range recorded values appear for review, without an automatic diagnosis.
- Review patient check-in concerns, record clinic action, and seek the patient’s confirmation of resolution. A “still need help” reply reopens the concern.
- Review clinic measures and service themes, or search PubMed metadata for a general topic and save sources to the clinic reading list.

How this helps the doctor

Fewer separate screens for queue, history and follow-through; clearer ownership of unfinished work; source-linked information before each visit; and explicit review points before clinical material reaches a patient. These are workflow benefits to validate in a clinic pilot, not measured time savings.

04 How an admin and receptionist use it

Admin: set up once, maintain as the clinic changes

1. Clinic details	Enter public facts such as hours, contact information, doctor schedules, holidays, standard patient questions and privacy settings.
2. Team	Create active doctor profiles and separate doctor/receptionist logins. The account role determines access.
3. Assistants	Configure routine assistant behavior and review channel readiness. Clinic factual settings inform responses; the standard booking conversation is product-owned.
4. Connections	Configure and inspect WhatsApp, phone and other software connections, including status and diagnostics.
5. Messages	Review and enable approved patient check-in invitation settings where a live channel and required template support exist.

Admin configures the system but cannot sign clinical documents or access doctor-only clinical endpoints through the admin role.

Receptionist: supervise the live front desk

- See today’s appointments and queue, manually check in a patient, help with holds/no-shows and handle conversations routed to a person.
- Respond to operational flags, monitor patient replies and help rebook when a schedule changes.
- Manage cancellation waiting-list opt-in and offers; accepted offers recheck consent, identity and slot availability.
- Own callbacks and patient check-in contact history. Clinical decisions and closure of clinical concerns return to a doctor.
- Handle privacy requests and consent changes in the designated workspace.

Doctor running late: example

The doctor enters “running 30 min late.” Sahayak computes which patients are affected. The doctor checks the impact and explicitly confirms. Only then does the system prepare notifications and possible new slots; reception watches replies and exceptions.

05 Capabilities: front desk and patient communication

Capability	What it does
Appointments	First-visit/follow-up menu, real doctor availability, final confirmation, booking changes/cancellations and schedule impact review.
Check-in and queue	QR/link check-in with consent, token issuance, live queue, waiting-room screen, call next, hold/skip and wait estimates.
Conversations	English/Hindi clinic information, human handoff, conversation history and staff oversight. A local WhatsApp-like simulator supports demos.
Safety	Deterministic Hindi/English red-flag phrases plus optional model check, fixed emergency response, urgent clinic alert and medical-advice refusal.
Reminders and schedule	Consent-aware reminder concepts, doctor-approved late-running impact, affected-patient notifications and rebooking flow in the supported channel path.
Waiting list	Consent-based opt-in, ordered cancellation offers, 15-minute slot hold, identity/expiry checks and atomic acceptance.
Patient check-ins	Agreed post-visit schedule, private Hindi/English form, recovery and clinic-experience responses, staff ownership, action history and patient resolution confirmation.
Feedback and improvement	Source-linked themes, response/contact counts and assigned clinic improvement tasks. Silence is not treated as satisfaction.

Channel status, stated plainly

The local simulator and clinic software workflows are available for demonstrations. WhatsApp messaging has assisted pilot setup and a deployed bridge, but connection/readiness does not by itself prove delivery of a fresh patient message. Scheduled check-in invitations need an approved supported template and a public HTTPS form. Phone/calling has bridge and readiness controls, but the documented pilot number is not authorized for WhatsApp Calling and no real call has been verified.

06 Capabilities: clinical work and follow-through

Capability	What it does
Doctor inbox	One prioritized view of alerts, conversations, overdue care work, reports, drafts and insurance work, with owner, acknowledgement, deferral and links to source records.
Consultation note	Typed/browser-dictated source-preserving draft; structured sections; explicit doctor edit and signature. Missing facts stay blank.
Patient record	Dated visits, notes, uploaded records, verified medicines/allergies/diagnoses, source-linked search and a pre-visit brief.
Reports and results	Uploaded file review, selectable-PDF text extraction, unverified numeric import, matching-unit trends and outside-range review tasks. No image interpretation.
Instructions	Draft from signed note, doctor edit and approval, fixed approved copy and download. Optional translation drafts in nine language choices need another review.
Follow-up plans	Versioned clinic templates, patient-specific edits, approval, step owner/date, pause/cancel, reply history and no-response review. Contact is currently recorded manually.
Prescriptions and checks	Doctor-controlled drafts with exact-name duplicate and verified allergy/active-medicine matches. These are limited prompts, not full drug interaction checking.
Clinic operations	Insurance case tracking; charges/payments already received; activity and backlog measures with defined denominators. No insurer submission or money movement.
References	PubMed publication metadata and source links for general topics, plus a dated clinic reading list. No generated patient-specific treatment recommendation.

Every clinical summary or imported value needs source review. Unverified extraction never silently becomes a verified fact.

07 How to make it more automated

The strongest next step is to connect already approved workflows to reliable live delivery and response handling. Keep doctor approval at the points where clinical judgment is required.

Stage	Practical implementation
Now: local workflow	Clinic staff configure facts and consent. Software records bookings, queue, notes, plans, tasks and alerts. Some messages are simulated or prepared; follow-up contact is often recorded by staff.
Next: live transport	Deploy stable public HTTPS services; verify WhatsApp templates, number ownership and real send/receive; capture provider delivery receipts; add retries, quiet hours and failure queues.
Then: closed loop	Trigger approved invitations and reminders from due dates. Match patient replies to the right appointment/check-in/plan. Create staff work for no reply, worsening symptoms or delivery failure.
Later: richer assistance	Offer draft summaries, translations and task suggestions with visible sources and doctor approval. Add OCR and more capable medication checks only after validated data sources and clinical evaluation.

Automation rules worth keeping

- **Consent before outreach:** distinguish booking, messaging, reminder and follow-up permissions; honor STOP and withdrawal.
- **Explicit approval:** doctor confirms schedule changes, signs notes and approves clinical instructions/plans before release.
- **Reliable execution:** use a stable request ID so repeated events cannot create duplicate bookings or messages; record delivery and errors.
- **Human exception queue:** uncertain delivery, urgent content, ambiguous replies and unresolved concerns go to an assigned staff member.
- **Measure the loop:** track due, prepared, sent, delivered, replied, reviewed and closed states; then measure real clinic time saved against a baseline.

Concrete first automation milestone

Choose one clinic and one consented workflow: approved post-visit check-in invitations. Prove actual delivery to test patients, private form completion, urgent-response escalation, staff follow-up, patient confirmation and opt-out. Expand only after the full loop is observable.

08 Safety, boundaries and handoff

Built-in controls

- Clinic data is separated by role and clinic. Staff actions and assistant tools are permission checked and audited.
- Patient-facing messages support Hindi and English in the core front-desk flow. Consent is recorded; private form links are short-lived and token-protected.
- Red-flag content bypasses routine automation and creates an urgent human alert. The assistant does not give medical advice.
- Model output is assistive. Notes, instruction translations and response classification retain original text for human review.
- If the model is unavailable, deterministic check-in, queue and basic software flows continue.

What Sahayak does not claim today

- No autonomous diagnosis, treatment recommendation, complete triage or clinically validated prescription interaction engine.
- No general OCR/image/ECG interpretation; selectable PDF text and manually entered values remain unverified until reviewed.
- No automatic delivery of local follow-up plans or proof that every configured live channel works end to end.
- No insurer submission, bank reconciliation, automatic refund or proven return on investment from existing activity metrics.

Suggested stakeholder walkthrough

1. Show Admin setup and role separation. 2. Book in the simulator. 3. Check in using a patient QR page. 4. Watch the queue and call the patient. 5. Open the doctor pre-visit brief and sign a note. 6. Approve instructions and a follow-up plan. 7. Show the inbox/feedback loop and a human handoff. 8. Close with the automation milestone above.

Source basis: Sahayak README; docs/clinic-workspaces.md; docs/CLINIC_APPOINTMENT_FLOW.md; docs/care-workflows.md; docs/local-workflows.md; docs/product-roadmap.md (5 Oct 2026 audit); docs/patient-checkins-plan.md; docs/WHATSAPP_ASSISTED_SETUP.md; docs/CALLING_PLATFORM.md; and the current workspace navigation.

Product description for discussion. External channel readiness and clinical use require clinic-specific validation.

09 From automatic booking to the doctor

The appointment begins in the patient conversation. It is not a task the doctor has to enter by hand.

Moment	What Sahayak does	What the doctor sees or does
Patient asks for a visit	Asks first visit/follow-up, captures required name and visit-data consent, shows real slots and saves only after the patient confirms YES.	The confirmed appointment appears in the doctor Today appointment list with time, patient, reason and status. The doctor need not copy a chat into a diary.
Before arrival	Keeps the booking in the clinic schedule; optional reminder outreach is a separate consent-based flow.	The doctor can review the day’s bookings and attention summary. No claim of an automatic push alert for every new booking.
Check-in	Patient or front desk checks in; a token and queue entry are created. Intake can collect the standard questions.	The doctor sees the waiting list, estimated wait, current patient and available intake summary.
Call and consult	Calling next updates the live queue and waiting display.	The doctor opens the source-linked patient brief, starts the consultation and documents the visit.
Finish and follow up	The visit checklist surfaces outstanding notes, documents, instructions, plans and tasks.	The doctor reviews, signs or approves, assigns follow-up and finishes. New replies or due items come back to the inbox.

Doctor benefit at the handoff

The confirmed booking flows into the doctor’s working list. Check-in and intake add context before the patient enters the room. The doctor spends attention on the visit and clinical review, while Sahayak tracks the surrounding administrative work.

10 Morning to night: two people, many workflows

When	Doctor	One office operator (Admin + Reception)
Before opening	Reviews Today, appointments, urgent/overdue inbox work and patients who need follow-through.	Checks clinic hours, connection status and the day's queue. Handles schedule exceptions and changes factual settings if needed.
During arrivals	Calls the next patient and reads the brief as check-ins and intake arrive.	Watches the live queue; helps walk-ins or patients who cannot use QR; handles human conversations and urgent flags.
During consultation	Dictates/types, checks source records, signs notes and approves instructions; checks delays before notifying affected patients.	Takes operational calls and rebooking exceptions; watches waiting-list offers and unresolved messages.
After each visit	Uses the completion checklist; assigns tasks, an approved plan or an agreed patient check-in.	Tracks callback ownership and practical patient questions. Clinical decisions remain with the doctor.
End of day	Reviews unsigned work, due follow-ups, unresolved alerts, report tasks and the next day's schedule.	Reconciles open conversations, no-shows, pending offers and delivery failures; prepares tomorrow's roster and checks settings.

How one person covers the office

The goal is one office operator supervising automated routine work. Today, Admin and Receptionist are separate software roles. The same person can hold separate accounts and switch between them: Admin for setup and connections, Receptionist for queue and conversations. This is an operating model, not a claim that one admin login currently has both permission sets.

What gets automated, and what remains human

Booking and queue state can move through the software automatically; configurable reminders and event-based assistants can be drafted, previewed and activated, subject to channel readiness and consent. The operator handles uncertain messages, delivery failures, walk-ins and urgent issues. The doctor owns diagnoses, clinical approvals and final decisions.

For the full role procedures, see the companion PDFs: Sahayak One-Person Admin Flow and Sahayak Doctor Day Flow. Source basis: current Today, Admin, Reception and visit-checklist screens; docs/CLINIC_APPOINTMENT_FLOW.md; docs/clinic-workspaces.md; docs/local-workflows.md.